Docetaxel 75 mg/m² · every 3 weeks

Docetaxel 75 mg/m² by IV infusion on Day 1 every 21 days. For the source breast indication, continue until progression or unacceptable toxicity; the team reviews benefit and tolerability. The six-cycle source maximum applies to NSCLC.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hands / feet

Docetaxel

Report numbness or tingling before the next dose.

2 Blood / infection

Chemotherapy phase

Fever, shivers or suddenly unwell: call immediately.

3 Hair / scalp

Chemotherapy phase

Hair loss is common. Ask about scalp cooling before treatment where suitable.

4 Fluid / skin / nails

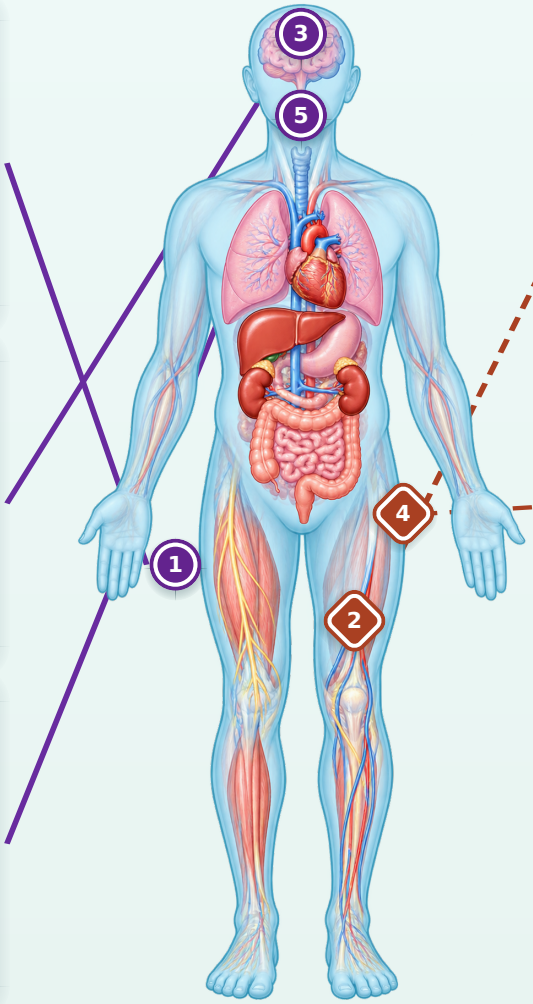
Docetaxel

Report swelling, painful skin or nail changes.

5 Mouth

Chemotherapy phase

Painful ulcers or difficulty drinking need advice.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects; the full risk discussion includes hair loss, fertility and other individual risks.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Docetaxel

- Low blood counts, infection and sepsis
- Hair loss, mouth soreness and fatigue
- Numbness / tingling; sometimes lasting
- Fluid retention and nail changes
- Allergic infusion reaction

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

The correct phase prescription

Taxanes use low-risk antiemetic support; hypersensitivity premedication is a separate requirement.

Premedicines and growth factors

Docetaxel dexamethasone is taken for three days starting the day before treatment. Ask the team to reconcile this with antiemetic steroids.

Tests and appointments

The team should name the booking owner, due window and person reviewing each required test. Tell them if an expected appointment has not arrived.

After chemotherapy

Discuss recovery, fertility, rehabilitation and the next treatment or surveillance step.

Review the full medicine plan

Dexamethasone 8 mg orally twice daily for 3 days starting the day before docetaxel; at least 3 doses before treatment unless contraindicated. Missed doses require contact; 20 mg IV replacement is consultant-directed. Reconcile the complete steroid plan with the antiemetic plan; do not add duplicate dexamethasone. Low emetic risk: separate prevention/rescue medicines according to the national/local plan. Prophylactic G-CSF may be used after individual risk assessment.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00203/

Sources and review status are available online. Follow your own prescription and hospital instructions.